

D20 — Medicine Attending Note ★

Dr. M. Torres, Medicine Attending | Medicine | 02/26/2026 09:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

MEDICINE ATTENDING PROGRESS NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156

Date: 02/26/2026 09:00 | HD #9 | Provider: Dr. M. Torres, Medicine Attending

LABS (AM): Na 138 | K 4.6 | Cr 1.9 (stable x 48h) | Hgb 8.5 | Glucose 220

Patient clinically doing well. Tolerating regular diet. No melena or hematemesis x 4 days. Ambulating with assist.

ADDRESSING CONFLICTING SPECIALIST RECOMMENDATIONS:

I have reviewed the cardiology (Dr. Kim) and nephrology (Dr. Gupta) reconsult notes from yesterday. Their recommendations differ on two key medications:

LISINOPRIL:

- Cardiology: Restart at 10 mg daily. CHF mortality benefit outweighs risk.
- Nephrology: HOLD until Cr $\leq$ 1.6.
- MY DECISION: Will restart lisinopril at 10 mg daily. Cr has been stable at 1.9 for 48 hours with a downward trend. The mortality benefit of ACEi in HFrEF is substantial. Will monitor Cr closely – if rises $>$ 0.3 from restart value, will hold.

SPIRONOLACTONE:

- Cardiology: Restart at 25 mg daily. Mortality benefit.
- Nephrology: HOLD until K⁺ $<$ 4.5 AND Cr at baseline.
- MY DECISION: Will HOLD spironolactone at discharge. K⁺ 4.6 today with AKI not yet resolved (Cr 1.9, baseline 1.5). The risk of hyperkalemia is too high at this time. Agree with nephrology. Restart as outpatient once K⁺ consistently $<$ 4.5 AND Cr at or near baseline. Will communicate this to cardiology heart failure clinic.

OTHER MEDICATION DECISIONS:

- Furosemide: Restart 20 mg PO daily per cardiology.
- Allopurinol: Restart 100 mg PO daily (Cr stable, dose appropriate for eGFR).
- Apixaban: Remains held per IR. Earliest restart 02/27 (Day 10).
- Prednisone: Currently 20 mg. Taper plan: 20 mg x 3 more days (through 03/01), then 10 mg x 4 days (03/02-03/05), then 5 mg x 3 days (03/06-03/08), then stop.
- Bowel regimen: senna scheduled + docusate + MiraLAX. Constipation improving.
- Endocrinology consulted for DM management.

Target discharge: Day 11 (02/28) pending IR clearance for anticoagulation restart and stable labs.

Electronically signed by: Dr. M. Torres, MD

Date: 02/26/2026 09:45

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